

ENGLISH FOR WORK / SEPTEMBER 2026

Healthcare Administration English



Learner workbook

Work with realistic cases, develop your own response, and use the answer section after you have tried the tasks.

PRACTICE THAT TRANSFERS TO WORK

Read a realistic case. Study complete conversations. Find the right language, speak, get feedback, and try again.

Eight lessons | Intermediate to advanced | Independent or classroom study

For hospital administrators, clinic managers, practice administrators, care coordinators, patient-experience leaders, revenue-cycle staff, operations managers, and healthcare-adjacent professionals.

Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

All scenarios, figures, and model responses are fictional teaching material. Use invented details in practice.

[Open this course on English Ladder](#)

Your course at a glance

Use the lesson numbers to match this document with the website and the other guides.

01 Patient Access, Scheduling, and Referrals

Clarify missing information before responding.

02 Revenue Cycle, Coding, and Denials

Explain a useful distinction in plain English.

03 Patient Flow, Capacity, and Staffing

Distinguish completed work from pending work and explain its impact.

04 Quality, Safety, and Accreditation

Separate observations, assumptions, and conclusions.

05 HIPAA, Privacy, and Information Governance

Make a request with a clear action, purpose, and realistic timing.

06 Patient Experience and Service Recovery

Acknowledge a communication problem and give a corrected message.

07 Population Health and Care Coordination

Give a handoff that the receiving person can confirm.

08 Executive Dashboards and Board Updates

Explain a useful distinction in plain English.

Choose your pace

Quick practice: spend 15 minutes reading one conversation and rehearsing one scenario. Full lesson: allow 45-60 minutes for language work, three conversations, two speaking scenarios, and feedback.

Level support

B1 learners can use the frames and a partner. B2 learners can try a response before reading the model. C1 learners can add the harder follow-up and reduce preparation time. These are teaching suggestions, not certified CEFR level ratings.

After practicing: open the AI prompt workshop, or use the prompts at the end of this guide.

LESSON 01 / READ AND NOTICE

Patient Access, Scheduling, and Referrals

Clarify missing information before responding.

SITUATION

A referral is received without the requested specialty or reason for the appointment. The patient is expecting a booking confirmation.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

referral - Order, recommendation, or routing process that directs a patient or case to another clinician, service, or specialist.

eligibility - Rules that determine whether a person, account, provider, service, or request qualifies for a program, benefit, or action.

authorization - Required approval from a payer, regulator, manager, or system before a service, transaction, or action can proceed.

no-show - Scheduled patient, customer, or guest who does not arrive or cancel in time for the appointment or reservation to be reused.

3. Notice the language

Name the exact word, fact, or requirement you need clarified. Ask one focused question at a time. Repeat your understanding and give the other person a chance to correct it.

When you say ..., do you mean ...?

Could you clarify which ...?

So, my understanding is Is that right?

Compare this wording: Could you explain what does this mean?

LESSON 01 / PRACTICE AND PRODUCE

Ask a precise question

4. Check the language

1. Which indirect question has the usual word order?
 - A. Could you confirm when does the review start?
 - B. Could you confirm when starts the review?
 - C. Could you confirm when the review starts?
2. Someone says the report is 'ready.' Which question best clarifies its approval status?
 - A. Who will present the report at the meeting?
 - B. Has the report been approved, or is it ready for review?
 - C. Can you send the report to me this afternoon?

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 01 / 05 • CONVERSATIONS / 1 OF 3

A referral without a specialty

Fictional scheduling call: a referral lacks the requested specialty and appointment reason, while the patient expects confirmation.

01 • Access coordinator: This referral has no specialty or appointment reason. Could you clarify what service was requested?

02 • Referral clerk: I can check with the referring clinician, but I can't determine the specialty myself.

03 • Access coordinator: The patient expects a booking confirmation. Should I select a department from their previous visits?

04 • Referral clerk: No. That would substitute an assumption for the missing referral information.

05 • Access coordinator: I'll explain that we received the referral and are seeking the details needed for scheduling.

06 • Referral clerk: Please keep that separate from eligibility or authorization status, which we haven't checked here.

07 • Access coordinator: Can you ask for the specialty and reason, then return the confirmed information to me?

08 • Referral clerk: Yes. I'll contact the referring team and identify your request as a scheduling clarification.

09 • Access coordinator: I'll remain the contact for the booking update without promising an appointment time.

10 • Referral clerk: Understood. I'll send the clarification through the designated referral channel once the team responds.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 01 / 05 • CONVERSATIONS / 2 OF 3

Eligibility checked, authorization still unknown

Separate fictional access case: an eligibility check is complete for a scheduled service, but authorization status is blank.

01 • Scheduling supervisor: The eligibility check is complete. Can we tell the department authorization is confirmed too?

02 • Authorization specialist: No. Those fields describe different checks, and the authorization status is currently blank.

03 • Scheduling supervisor: Does blank mean authorization is unnecessary for this appointment, or that it was denied?

04 • Authorization specialist: Neither conclusion is supported. We need the applicable requirements and the case status verified.

05 • Scheduling supervisor: Which details do you need to perform that check without requesting the entire patient record?

06 • Authorization specialist: Please provide the service reference and scheduled date through our authorized work queue.

07 • Scheduling supervisor: Can you confirm which status wording I should use while your check is pending?

08 • Authorization specialist: Say eligibility checked, authorization status not yet confirmed. Avoid any promise about payment.

09 • Scheduling supervisor: I'll use those separate labels and direct follow-up questions to your queue.

10 • Authorization specialist: I'll return the verified status and identify any additional information needed before the scheduling team responds.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 01 / 05 • CONVERSATIONS / 3 OF 3

A cancelled visit counted as a no-show

Separate fictional appointment review: a cancellation message arrived before the visit, but the appointment is marked no-show; receipt time is recorded.

01 • Patient access analyst: This visit is marked no-show, but a cancellation message arrived before the appointment.

02 • Scheduling lead: Have you checked whether the message refers to this visit rather than another booking?

03 • Patient access analyst: The appointment reference matches. I haven't checked how the status was entered.

04 • Scheduling lead: Then let's clarify the workflow before assuming someone ignored the cancellation message.

05 • Patient access analyst: Could this affect our reported no-show count and the patient's future contact?

06 • Scheduling lead: It could. We should verify the appropriate classification under our scheduling process.

07 • Patient access analyst: Will you review the message timestamp and appointment history with the responsible team?

08 • Scheduling lead: Yes. Please keep the original record available so any correction has a clear basis.

09 • Patient access analyst: I'll flag this entry for review rather than silently changing the dashboard total.

10 • Scheduling lead: I'll confirm the outcome and make sure the access team receives the corrected status if needed.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 01 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Patient Access, Scheduling, and Referrals

A referral is received without the requested specialty or reason for the appointment. The patient is expecting a booking confirmation.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You need to know exactly what information is missing. Give one answer only after your partner asks a focused question; then ask them to confirm their understanding.

Second round: The other person uses an ambiguous word such as 'ready', 'approved', or 'urgent'. Clarify it before continuing.

Model for scenario 1: Could you confirm the specialty and referral details through the approved channel? I can explain the scheduling process to the patient, but I cannot confirm an appointment until the required information is available.

Scenario 2 | Two referral entries, one uncertain request

A fictional scheduling queue contains two referrals with the same patient reference and date but different originating departments. Neither entry says whether it replaces the other. Discuss what must be clarified before booking.

Role A: Referral coordinator: ask whether these are duplicate or separate requests.

Role B: Referring-office administrator: verify the intended requests with the clinical team without choosing a specialty yourself.

Possible opening: These entries may be duplicates, but the different departments mean I need confirmation before booking.

Success checks

- Do not infer duplication from matching dates alone.
- Ask which request remains active and what service is intended.
- Agree who returns the verified scheduling instruction.

Add a complication: A colleague proposes deleting the later entry to shorten the queue.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

LESSON 02 / READ AND NOTICE

Revenue Cycle, Coding, and Denials

Explain a useful distinction in plain English.

SITUATION

A dashboard groups rejected submissions and denied claims together. A manager wants to understand where follow-up work is needed.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

claim - A statement that may need evidence, approval, qualification, or disclosure before it is used externally.

coding - Assignment of standardized diagnostic, procedure, or service codes used for records, billing, reporting, or reimbursement.

denial - Decision by an insurer, payer, regulator, or authority not to approve, pay, or grant a requested item or claim.

clean claim - Insurance claim submitted with the required data and documentation so it can be processed without avoidable delay or rejection.

3. Notice the language

Start with the reader's question. Explain one unfamiliar term with familiar words, then give a relevant example or contrast. Check understanding without asking only 'Do you understand?'.

In this context, ... means

The difference is that

How would you explain that distinction to a colleague?

Compare this wording: We need to operationalize the implementation.

LESSON 02 / PRACTICE AND PRODUCE

Make a complex point clear

4. Check the language

1. Which sentence explains 'backlog' rather than repeating it?
 - A. The backlog is the maximum work the team can finish in a period.
 - B. The backlog is the work still waiting to be completed.
 - C. The backlog is the work completed during the last period.
2. Which follow-up best checks whether your explanation was clear?
 - A. How would you describe the next step in your own words?
 - B. Would you like me to repeat the explanation?
 - C. Was the explanation detailed enough for you?

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 02 / 05 • CONVERSATIONS / 1 OF 3

Rejected submissions and denied claims need different queues

Fictional revenue-cycle review: the dashboard combines submissions rejected before processing and claims denied after review.

01 • Operations manager: Why can't we assign every item in this dashboard to the denial team?

02 • Revenue cycle analyst: It combines rejected submissions with denied claims, although their follow-up paths may differ.

03 • Operations manager: Could you explain that distinction without assuming I know the processing terminology?

04 • Revenue cycle analyst: In this dataset, rejected items didn't enter processing; denied claims received an adverse payment decision.

05 • Operations manager: So correcting a submission field isn't necessarily the same task as reviewing a denial reason?

06 • Revenue cycle analyst: Exactly. We need the actual response codes and local workflow before assigning each item.

07 • Operations manager: Does a corrected submission automatically become a paid claim once it is accepted?

08 • Revenue cycle analyst: No. Acceptance for processing doesn't guarantee payment or settle coding questions.

09 • Operations manager: I'll ask for separate totals and owners rather than one undifferentiated problem count.

10 • Revenue cycle analyst: I'll split the queue by verified status and retain items whose response codes still need interpretation.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 02 / 05 • CONVERSATIONS / 2 OF 3

A clean claim is not a payment guarantee

Separate fictional billing meeting: twenty submissions pass the team's completeness checks, but payer outcomes are not available.

01 • Billing supervisor: Twenty claims passed our checks. May I describe them as twenty confirmed payments?

02 • Claims specialist: No. They passed completeness checks; we haven't received the payer outcomes yet.

03 • Billing supervisor: Then what does clean claim mean in this particular internal report?

04 • Claims specialist: It means the submissions met our specified checks for processing, not that payment was promised.

05 • Billing supervisor: Could there still be a coding or coverage question after those checks?

06 • Claims specialist: Yes, depending on the case. We should use actual responses rather than predict a denial or payment.

07 • Billing supervisor: How should I explain the progress to the department without overstating collections?

08 • Claims specialist: Say twenty checked submissions are ready for the next processing stage and outcomes remain pending.

09 • Billing supervisor: I'll separate submission quality from payment status in the meeting summary.

10 • Claims specialist: I'll confirm the response status later and keep the report's clean-claim definition attached to the metric.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 02 / 05 • CONVERSATIONS / 3 OF 3

Denial count and denial value tell different stories

Separate fictional dashboard exercise: ten denied claims total 1,000 units of currency; two other denied claims total 8,000, with no recovery estimates supplied.

01 • Finance partner: The first group has more denials. Should it automatically receive all our follow-up capacity?

02 • Denials analyst: Not necessarily. Ten claims total one thousand, while the other two total eight thousand.

03 • Finance partner: So the count measures workload, while the amount shows the value involved?

04 • Denials analyst: Those are useful distinctions, although complexity and recoverability also matter and remain unknown here.

05 • Finance partner: Can we assume the larger amount is entirely recoverable if we act quickly?

06 • Denials analyst: No. We need the denial reasons and supporting records before estimating an outcome.

07 • Finance partner: What should the team compare before proposing a work order for these groups?

08 • Denials analyst: Claim count, value, verified deadlines, and case complexity, with unknowns clearly labeled.

09 • Finance partner: I'll avoid ranking the groups by one headline number and ask for that assessment.

10 • Denials analyst: I'll bring a scoped comparison without converting the denied amount into a promised recovery.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 02 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Revenue Cycle, Coding, and Denials

A dashboard groups rejected submissions and denied claims together. A manager wants to understand where follow-up work is needed.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You are unfamiliar with one technical term in the case. Ask for a plain-English explanation and then paraphrase it. Do not pretend to understand a term you cannot explain.

Second round: Your listener is new to this field. Replace two specialist expressions with clear explanations without changing the meaning.

Model for scenario 1: These categories describe different points in the process. Let's separate submissions that did not enter processing from claims that received a denial, then identify the follow-up required for each group.

Scenario 2 | A returned claim and a suspected coding problem

A fictional submission was returned for a missing identifier. A manager calls it a coding denial, but no adjudication response or coding review exists. Explain the known processing issue and route the next action.

Role A: Billing analyst: distinguish the returned submission from an established coding denial.

Role B: Department administrator: ask what needs correcting and what remains unassessed.

Possible opening: The available response identifies a missing identifier; it does not establish a coding denial.

Success checks

- State the observed response without adding a coding conclusion.
- Separate correction of submission data from review of clinical coding.
- Agree the appropriate queue and a verified status label.

Add a complication: The manager wants the item included in a coding-error presentation that afternoon.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

LESSON 03 / READ AND NOTICE

Patient Flow, Capacity, and Staffing

Distinguish completed work from pending work and explain its impact.

SITUATION

A unit reports 24 occupied beds and two planned discharges. The discharge timing is unconfirmed, but another team assumes two beds are available now.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

census - The number of patients counted in a facility or unit at a defined time.

bed management - Coordinating hospital bed availability and assignment in relation to patient needs.

throughput - Amount of work, patients, goods, cases, or transactions completed in a period of time.

staffing ratio - The relationship between staff numbers and a defined workload or population.

3. Notice the language

Lead with the current state, then the important change or open item. Use completed-action language only for completed work. Add a next update point when it is known; do not invent a date to make the message sound complete.

We have completed ...; ... remains open.

As of ..., the status is

The next update will cover

Compare this wording: Everything is progressing well.

LESSON 03 / PRACTICE AND PRODUCE

Give a useful status update

4. Check the language

1. Which sentence says a review is finished?
 - A. The team has completed the review.
 - B. The team is completing the review.
 - C. The team plans to complete the review.
2. Which update distinguishes a plan from a confirmed fact?
 - A. Delivery is confirmed for Friday because we planned it.
 - B. The delivery plan proves that arrival will be Friday.
 - C. Delivery is planned for Friday; the carrier has not confirmed it.

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 03 / 05 • CONVERSATIONS / 1 OF 3

Two planned discharges are not two available beds

Fictional capacity call: the unit has 24 occupied beds and two planned discharges whose timing is unconfirmed.

01 • Bed manager: Your unit expects two discharges. Can I show two beds available right now?

02 • Unit administrator: No. Our current census is twenty-four occupied beds, and both discharge times are unconfirmed.

03 • Bed manager: The transfer team interpreted planned as completed. What should I tell them instead?

04 • Unit administrator: Say two discharges are anticipated, but current bed availability has not changed.

05 • Bed manager: Would departure alone establish that a bed is ready for the next patient?

06 • Unit administrator: We also need the relevant readiness checks under our local bed-management process.

07 • Bed manager: I'll keep the pending movements separate from confirmed capacity on the board.

08 • Unit administrator: Thank you. I'll check the status with the responsible clinical and operational teams.

09 • Bed manager: Please update me when a change is confirmed, including any staffing dependency.

10 • Unit administrator: I will. I won't turn an expected discharge into an available-bed promise before readiness is established.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 03 / 05 • CONVERSATIONS / 2 OF 3

Physical space without confirmed staffing

Separate fictional capacity exercise: a room is prepared, but staffing coverage for an additional admission has not been confirmed.

01 • Capacity coordinator: The room is ready. Why does the capacity board still show admission readiness pending?

02 • Staffing coordinator: Because staffing coverage for an additional admission has not yet been confirmed.

03 • Capacity coordinator: Could we count the room as staffed capacity and resolve the roster later?

04 • Staffing coordinator: That would hide an important dependency. Physical space and operational capacity aren't the same status.

05 • Capacity coordinator: Should I quote a standard staffing ratio to settle the question in this exercise?

06 • Staffing coordinator: No universal ratio is supplied here. The authorized team must assess the applicable staffing requirements.

07 • Capacity coordinator: I'll report room preparation complete and staffing confirmation pending, with no admission promise.

08 • Staffing coordinator: I'll ask the responsible supervisor for the coverage assessment and expected update time.

09 • Capacity coordinator: Can you return a confirmed status directly to bed management when you receive it?

10 • Staffing coordinator: Yes. I'll close that loop so the board reflects usable capacity rather than room count alone.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 03 / 05 • CONVERSATIONS / 3 OF 3

A throughput update with an unresolved transport step

Separate fictional flow exercise: discharge paperwork is complete for one planned departure, but transport arrival has not been confirmed.

01 • Flow manager: Is the discharge complete now that the paperwork is finished for this case?

02 • Discharge administrator: Only the paperwork step is complete. Transport arrival is still unconfirmed.

03 • Flow manager: The throughput report needs a status before the afternoon call. What can we safely report?

04 • Discharge administrator: Administrative preparation complete, departure pending transport confirmation. That states the completed work and the blocker.

05 • Flow manager: Can we use the requested collection time as the actual departure time?

06 • Discharge administrator: No. A requested time is not an observed departure or a carrier confirmation.

07 • Flow manager: Who is following up so the receiving team doesn't make a duplicate request?

08 • Discharge administrator: I'll contact the transport coordinator and remain responsible for the administrative status update.

09 • Flow manager: Please distinguish the booking status from any clinical decision about discharge readiness.

10 • Discharge administrator: Understood. I'll report the operational facts and refer clinical readiness questions to the responsible clinician.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 03 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Patient Flow, Capacity, and Staffing

A unit reports 24 occupied beds and two planned discharges. The discharge timing is unconfirmed, but another team assumes two beds are available now.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You need to brief someone who has only one minute. Ask what is complete, what is open, and which open item affects the next action.

Second round: The listener asks you to reduce the update to two sentences. Preserve the most important completed and pending items.

Model for scenario 1: There are 24 occupied beds. Two discharges are planned, but their timing is not confirmed. I'll distinguish current availability from expected availability in the capacity update.

Scenario 2 | A census snapshot taken at a different time

In a fictional morning briefing, the overnight report shows eighteen occupied beds at 06:00 and the live board shows twenty at 08:00. Movement records have not been reconciled. Discuss the update without treating either count as an error.

Role A: Capacity analyst: explain the snapshot times and request reconciliation.

Role B: Unit coordinator: identify the need to check movements before updating the executive report.

Possible opening: These counts were taken two hours apart, so the difference does not by itself establish an error.

Success checks

- State both counts with their times.
- Separate a possible movement difference from a confirmed data error.
- Agree who verifies the current census before it is circulated.

Add a complication: The chair asks you to use the lower number because it looks better.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

LESSON 04 / READ AND NOTICE

Quality, Safety, and Accreditation

Separate observations, assumptions, and conclusions.

SITUATION

A near-miss report identifies a scheduling error caught before the appointment. The review has not established how the error occurred.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

near miss - An event that could have caused harm but did not result in the specified harmful outcome.

accreditation - External recognition that an organization meets defined quality, safety, or professional standards.

audit trail - A record showing who changed what, when, and often why.

corrective action - Action taken to fix a current problem and prevent recurrence.

3. Notice the language

Say what the evidence shows, identify what is still unknown, and limit the conclusion to that evidence. Words such as 'may', 'suggests', and 'in this sample' are useful when the uncertainty is real. Do not soften an urgent, verified safety concern.

The available evidence shows

This may indicate ..., but

We have not yet established whether

Compare this wording: The pilot proves this will work everywhere.

LESSON 04 / PRACTICE AND PRODUCE

Match certainty to evidence

4. Check the language

1. Which sentence preserves uncertainty about an unconfirmed cause?
 - A. The change must have caused the delay.
 - B. The change definitely caused the delay.
 - C. The change may have contributed to the delay.
2. A result comes from a small pilot in one location. Which phrase accurately limits the claim?
 - A. Across the service, the observed result was ...
 - B. In this pilot, the observed result was ...
 - C. In similar locations, we can now expect the same result ...

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 04 / 05 • CONVERSATIONS / 1 OF 3

An intercepted scheduling error with no established cause

Fictional safety review: a scheduling error was caught before the appointment, and the cause remains unknown.

01 • Quality coordinator: The near miss involved a scheduling error caught before the appointment took place.

02 • Service manager: Can the report say a distracted scheduler caused it so we can assign corrective action?

03 • Quality coordinator: We haven't established that. The current report describes the error and its interception.

04 • Service manager: What evidence would help us understand how the incorrect booking entered the system?

05 • Quality coordinator: The audit trail, source request, and relevant staff accounts could help establish the sequence.

06 • Service manager: Should we leave the report open while those sources are being checked?

07 • Quality coordinator: Yes. We can record known facts without presenting a suspected cause as a finding.

08 • Service manager: I'll ask the team to preserve the records and avoid blame in the briefing.

09 • Quality coordinator: I'll distinguish immediate containment from any corrective action that depends on the review.

10 • Service manager: Then we'll discuss the verified sequence before deciding what process change is appropriate.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 04 / 05 • CONVERSATIONS / 2 OF 3

Missing evidence is not proof the check never happened

Separate fictional accreditation preparation: three sampled files lack a documented administrative check; whether the checks occurred is unknown.

01 • Accreditation coordinator: Three sampled files lack documentation of the administrative check. What should our finding say?

02 • Department lead: Could we state that staff skipped all three checks and schedule retraining?

03 • Accreditation coordinator: We know the evidence is missing. We don't yet know whether the activity occurred.

04 • Department lead: But doesn't the missing entry make this a confirmed documentation issue at least?

05 • Accreditation coordinator: Yes. We can describe the missing entries accurately and investigate the underlying process separately.

06 • Department lead: Would checking the audit trail help distinguish a recording problem from an omitted task?

07 • Accreditation coordinator: It may help. We should also ask the responsible team without suggesting the desired answer.

08 • Department lead: I'll keep the sample size visible so nobody assumes the finding covers every file.

09 • Accreditation coordinator: I'll request the supporting records and separate verified gaps from possible explanations.

10 • Department lead: We'll choose the corrective action after reviewing that evidence, not label retraining as an established solution today.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 04 / 05 • CONVERSATIONS / 3 OF 3

Fewer reports do not automatically mean fewer errors

Separate fictional quality meeting: near-miss reports fell from twelve to five after the reporting portal changed; reporting behavior has not been assessed.

01 • Operations director: Reports fell from twelve to five. Can I call the new portal a safety improvement?

02 • Quality analyst: Not yet. We know the report count changed, not whether underlying errors became less frequent.

03 • Operations director: What else could explain the difference besides safer work on the front line?

04 • Quality analyst: Reporting behavior may have changed, but that's a hypothesis we still need to assess.

05 • Operations director: So we should avoid saying the portal caused either better safety or underreporting?

06 • Quality analyst: Exactly. Both would go beyond the available evidence in this case.

07 • Operations director: What follow-up would give the review a clearer basis for interpreting the count?

08 • Quality analyst: Check comparable periods, portal access issues, and staff experience with submitting near misses.

09 • Operations director: I'll present the numbers as a reporting trend and keep the safety conclusion open.

10 • Quality analyst: I'll bring the follow-up evidence and distinguish the number of reports from the number of events.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 04 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Quality, Safety, and Accreditation

A near-miss report identifies a scheduling error caught before the appointment. The review has not established how the error occurred.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You need to tell a colleague what is known. Ask which part is confirmed and which part is an assumption. Challenge one statement that sounds more certain than the case supports.

Second round: Someone asks for a yes-or-no answer when the evidence supports only a qualified answer. Be concise while preserving the uncertainty.

Model for scenario 1: The error was identified before the appointment. The cause is still under review. I'll record the sequence of events and the checks that caught it without assigning blame or an unconfirmed cause.

Scenario 2 | A repeated error with different booking routes

Two fictional scheduling errors occurred this week, one through an online form and one by telephone. The final wrong department was the same. Discuss what can be concluded before a common root cause has been assessed.

Role A: Quality reviewer: state the shared outcome and different routes.

Role B: Scheduling manager: propose evidence gathering without declaring a single cause.

Possible opening: The errors reached the same department, but they entered through different routes.

Success checks

- Separate a common outcome from a demonstrated common cause.
- Request the relevant audit trails and source instructions.
- Keep corrective-action effectiveness unconfirmed until reviewed.

Add a complication: A colleague wants to close both cases under a single untested corrective action.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

LESSON 05 / READ AND NOTICE

HIPAA, Privacy, and Information Governance

Make a request with a clear action, purpose, and realistic timing.

SITUATION

A colleague requests a full patient list for a limited administrative task. The appropriate scope and access have not been confirmed.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

HIPAA - US health privacy framework governing protected health information in covered health contexts.

protected health information - Individually identifiable health information protected by HIPAA when held or transmitted by covered entities or business associates.

minimum necessary - HIPAA principle requiring use or disclosure of only the minimum protected health information needed for the purpose.

business associate - Under HIPAA, certain persons or organizations performing covered functions or services involving protected health information.

3. Notice the language

State the action and the information needed. Explain why it matters and specify a deadline only when one is given or agreed. 'Could you' is polite; repeated apologies can obscure a legitimate request. Urgent situations may require a direct request.

Could you provide ... so that ...?

Please confirm ... by [agreed time].

If that timing is not possible, please let me know

Compare this wording: Please revert soonest with the necessary.

LESSON 05 / PRACTICE AND PRODUCE

Ask for an actionable next step

4. Check the language

1. What does 'Please send it by Thursday' normally mean?
 - A. Keep sending it regularly until Thursday.
 - B. Send it no later than Thursday.
 - C. Send it on Thursday, but not earlier.
2. Which request gives the recipient enough detail to act?
 - A. Could you confirm the document version needed for the review?
 - B. Could you confirm that the review is important?
 - C. Could you confirm that you received my earlier message?

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 05 / 05 • CONVERSATIONS / 1 OF 3

Request the task scope before the full patient list

Fictional administrative request: a colleague asks for a full patient list for a limited task, and access scope is unconfirmed.

01 • Clinic administrator: Can you send the full patient list so I can complete this administrative task?

02 • Information governance officer: Please clarify the task, intended recipients, and fields you need before we consider the request.

03 • Clinic administrator: Would sharing everything now be faster than checking each field with the team?

04 • Information governance officer: It could disclose information beyond the task. We need the appropriate scope and access confirmed.

05 • Clinic administrator: Are you saying HIPAA always prohibits using a patient list for administration?

06 • Information governance officer: No. I'm asking for a case-specific review, not making a blanket legal statement.

07 • Clinic administrator: I'll describe the purpose and request only the information needed for that purpose.

08 • Information governance officer: Please use the approved access-request channel and identify when the task must be completed.

09 • Clinic administrator: I'll submit that today and wait for a verified sharing route rather than email the list.

10 • Information governance officer: I'll route it for review and clarify any minimum-necessary questions under the applicable process.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 05 / 05 • CONVERSATIONS / 2 OF 3

A vendor requests a sample file

Separate fictional privacy exercise: an unverified external vendor requests a patient-data sample for a demonstration; no sharing approval exists.

01 • Procurement coordinator: The vendor wants a sample file for tomorrow's demonstration. Could we send a few patient rows?

02 • Privacy officer: Please don't send protected health information before the purpose, access, and sharing conditions are reviewed.

03 • Procurement coordinator: They called themselves a business associate. Does that statement settle the approval question?

04 • Privacy officer: No. A vendor's label isn't verification of the relevant arrangement or permitted data use.

05 • Procurement coordinator: Would a synthetic demonstration file avoid using real patient information while we clarify this?

06 • Privacy officer: That may meet the demonstration need. Confirm the file is genuinely synthetic and suitable for the task.

07 • Procurement coordinator: I'll ask the vendor which functions they need to demonstrate rather than offer live records.

08 • Privacy officer: Please also request their intended data handling and route the proposal through the responsible reviewers.

09 • Procurement coordinator: I'll make that request today and keep the real sample out of the exchange.

10 • Privacy officer: Good. I'll help identify the review owner without treating tomorrow's meeting as permission to disclose.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 05 / 05 • CONVERSATIONS / 3 OF 3

Remove identifiers without claiming anonymity

Separate fictional reporting exercise: a draft administrative extract omits names but retains patient numbers and detailed appointment histories.

01 • Reporting analyst: I removed the names. Can I send this extract to the wider planning group?

02 • Privacy specialist: Please request a sharing review first. Patient numbers and detailed histories still remain in the file.

03 • Reporting analyst: I thought removing names meant the information was no longer identifiable or protected.

04 • Privacy specialist: That conclusion isn't established. Other fields or combinations can still create privacy concerns.

05 • Reporting analyst: What should I provide so the reviewer can assess the request efficiently?

06 • Privacy specialist: Describe the planning purpose, recipients, required fields, and proposed delivery method without circulating the extract broadly.

07 • Reporting analyst: Could a grouped count meet the purpose instead of sharing individual appointment histories?

08 • Privacy specialist: Ask the planning owner to confirm that. A smaller output may be sufficient, but it still needs assessment.

09 • Reporting analyst: I'll request that confirmation and keep the source extract in its restricted location.

10 • Privacy specialist: I'll coordinate the review and distinguish reduced detail from any verified claim that the output is anonymous.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 05 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | HIPAA, Privacy, and Information Governance

A colleague requests a full patient list for a limited administrative task. The appropriate scope and access have not been confirmed.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You can help but need a clear request and its purpose. Ask what is required and whether the timing is fixed or negotiable.

Second round: The other person cannot meet the requested timing. Clarify an alternative without inventing authority to change a formal deadline.

Model for scenario 1: Could you specify the purpose and fields required? Let's confirm the permitted access and necessary information through our privacy process before sharing the list.

Scenario 2 | An old folder permission follows a new role

A fictional administrator has moved to another department but can still open a patient-report folder used in their previous role. No new access need has been established. Hold a call requesting an access review without opening additional records.

Role A: Department administrator: report the remaining access and request confirmation of the appropriate permission.

Role B: Access-control specialist: ask for the folder reference and new role through the approved channel.

Possible opening: My previous patient-report access still works, but I need you to review whether it fits my new role.

Success checks

- State the access issue without browsing unnecessary patient records.
- Request a role-specific permission review.
- Agree a responsible reviewer and avoid claiming a legal conclusion.

Add a complication: A coworker suggests retaining access in case it becomes useful later.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

LESSON 06 / READ AND NOTICE

Patient Experience and Service Recovery

Acknowledge a communication problem and give a corrected message.

SITUATION

A patient has called twice about a delayed response. The administrator can investigate the communication delay but cannot make a clinical decision.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

grievance - A formally raised concern or complaint handled through an established process.

service recovery - Actions taken to address a service failure and restore the customer's experience.

HCAHPS - US standardized patient-experience survey used by hospitals for public reporting and quality improvement.

patient experience - A person's experience of interactions and services across their care.

3. Notice the language

Name what was unclear or wrong, acknowledge its effect, and correct it. A brief apology can help, followed by a practical next step. Avoid explaining away the other person's reaction or promising an outcome you cannot control.

My earlier message did not make ... clear.

I'm sorry for The correct information is

Let me check that the revised explanation addresses

Compare this wording: I'm sorry if you failed to understand.

LESSON 06 / PRACTICE AND PRODUCE

Repair a misunderstanding

4. Check the language

1. Which apology takes responsibility for unclear wording?
 - A. I'm sorry my earlier wording was unclear.
 - B. I'm sorry the process took longer than expected.
 - C. I'm sorry we could not meet the original date.
2. What should follow an apology for an incorrect date?
 - A. A longer explanation that never states the correct date.
 - B. A new guaranteed date even if it is unverified.
 - C. The corrected date and the current confirmation status.

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 06 / 05 • CONVERSATIONS / 1 OF 3

Repairing a promised callback message

Fictional service-recovery review: a patient has called twice about a delayed response, and the administrator cannot make clinical decisions.

01 • Patient experience lead: The patient has called twice. What explanation did our team provide about the delayed response?

02 • Clinic administrator: We said someone would respond, but we didn't give a clear owner for follow-up.

03 • Patient experience lead: How can we acknowledge that communication problem without promising a clinical decision ourselves?

04 • Clinic administrator: I'll apologize for the unclear follow-up and explain that I can investigate the communication delay.

05 • Patient experience lead: The patient may ask whether the delayed response means their request was refused.

06 • Clinic administrator: I'll clarify that I don't have a confirmed clinical decision and won't infer one from the delay.

07 • Patient experience lead: What concrete service-recovery step can you personally take and explain accurately?

08 • Clinic administrator: I'll contact the responsible team and confirm who will provide the next update.

09 • Patient experience lead: Please distinguish that update from a promise that the underlying matter will be resolved.

10 • Clinic administrator: Agreed. I'll own the communication follow-up and use the appropriate grievance route if requested.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 06 / 05 • CONVERSATIONS / 2 OF 3

Correcting a guaranteed appointment message

Separate fictional patient-experience case: an administrator said an appointment was guaranteed, but the record shows only a requested slot.

01 • Scheduling manager: Your earlier message said guaranteed, but the record shows the appointment is only requested.

02 • Access administrator: I need to correct that clearly and acknowledge that my wording created the wrong expectation.

03 • Scheduling manager: What will you say without blaming the patient for misunderstanding the message?

04 • Access administrator: I said guaranteed when I should have said requested. The appointment is not yet confirmed.

05 • Scheduling manager: Will you promise another slot immediately to repair the patient experience?

06 • Access administrator: Not unless availability is verified. I'll check the request and explain the confirmed options.

07 • Scheduling manager: Please also acknowledge the practical inconvenience caused by the earlier message.

08 • Access administrator: I will, and I'll take responsibility for returning an accurate scheduling update.

09 • Scheduling manager: How will you make sure the next colleague doesn't repeat the original promise?

10 • Access administrator: I'll correct the status through our process and hand over the communication issue without erasing the original history.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 06 / 05 • CONVERSATIONS / 3 OF 3

A survey score is not this person's resolved concern

Separate fictional service review: a department cites an improved HCAHPS score while one patient's reported communication concern remains unresolved.

01 • Service director: Our HCAHPS score improved. Can we tell the patient that communication problems have been fixed?

02 • Patient experience specialist: No. The survey trend doesn't resolve this person's outstanding concern or explain what happened.

03 • Service director: How should I repair that message if I already mentioned the score as reassurance?

04 • Patient experience specialist: Acknowledge that the general result didn't address their experience, then return to their specific concern.

05 • Service director: Could I say their experience must have been unusual because the score improved?

06 • Patient experience specialist: That would dismiss the concern without investigating it. We don't know how representative it is.

07 • Service director: I'll say we still need to review the missed communication and confirm a follow-up owner.

08 • Patient experience specialist: Good. Explain the service-recovery or grievance route available through our organization without promising a finding.

09 • Service director: I'll separate the department metric from the individual's case in the conversation.

10 • Patient experience specialist: And I'll coordinate the case follow-up so the apology is accompanied by a clear next action.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 06 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Patient Experience and Service Recovery

A patient has called twice about a delayed response. The administrator can investigate the communication delay but cannot make a clinical decision.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You are frustrated because an earlier message created an expectation. Explain that expectation. Ask your partner to distinguish the correction from anything still uncertain.

Second round: The listener remains disappointed after the correction. Acknowledge the impact and restate the available next step calmly.

Model for scenario 1: I'm sorry you have had to call again. I can check the status of your request and confirm the next contact. For the clinical question, I'll connect you with the appropriate care team.

Scenario 2 | Directions to the wrong entrance

A fictional appointment reminder directed visitors to Entrance A, which is closed for this training case; the confirmed open entrance is B. A patient missed check-in while looking for A. Discuss a corrective phone response without promising clinical rescheduling.

Role A: Patient access supervisor: acknowledge the incorrect directions and provide the verified entrance information.

Role B: Patient liaison: ask how the appointment question will reach the appropriate scheduling team.

Possible opening: Our reminder gave the wrong entrance; Entrance B is the confirmed route, and I'm sorry for the inconvenience.

Success checks

- Accept the communication error without blaming the patient.
- Give the verified correction and distinguish it from rescheduling authority.
- Agree who reviews the appointment status and contacts the patient.

Add a complication: A colleague wants to describe the patient as a no-show without reviewing the circumstances.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

LESSON 07 / READ AND NOTICE

Population Health and Care Coordination

Give a handoff that the receiving person can confirm.

SITUATION

A discharge coordination note lists transport as pending. The next coordinator needs to know who is following up and what has been confirmed.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

care gap - A difference between recommended or intended care and the care a person has received.

readmission - Return of a patient to a hospital or facility within a defined period after discharge, often tracked as an outcome measure.

discharge planning - Coordinating the needs and arrangements for a patient leaving a care setting.

social determinants - Social and environmental conditions that influence people's health and access to opportunities.

3. Notice the language

State the current situation, relevant background, open task, and receiving role. Ask for acknowledgment or repeat-back of the critical item. Sending a message and transferring responsibility are not always the same thing.

The current status is ...; the outstanding item is

The record shows

Please confirm who has accepted

Compare this wording: Someone should follow up on this.

LESSON 07 / PRACTICE AND PRODUCE

Transfer information and responsibility

4. Check the language

1. Which sentence checks that responsibility has been accepted?
 - A. I mentioned the check in my earlier email.
 - B. The check appears somewhere in the notes.
 - C. Please confirm who will complete the outstanding check.
2. Which response best checks a critical handoff detail?
 - A. I saw the message, so no clarification is needed.
 - B. So the result is pending and your team will follow up; is that correct?
 - C. I assume all results are final, so we can finish.

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 07 / 05 • CONVERSATIONS / 1 OF 3

Transport pending at the coordination handoff

Fictional discharge-coordination handoff: the note lists transport as pending, with no confirmed collection time.

01 • Outgoing coordinator: Transport remains pending in this discharge-planning case; I don't have a confirmed collection time.

02 • Incoming coordinator: Who has been following up, and what information can I rely on now?

03 • Outgoing coordinator: The only confirmed status here is pending. I'll pass the request reference so you can verify ownership.

04 • Incoming coordinator: Please don't describe transport as booked unless the provider has actually confirmed it.

05 • Outgoing coordinator: Agreed. I'll distinguish the request from confirmation and avoid implying a clinical discharge decision.

06 • Incoming coordinator: I'll take ownership of the next administrative follow-up once I receive the reference.

07 • Outgoing coordinator: Can you read back the unresolved item so we know the handoff is clear?

08 • Incoming coordinator: Collection timing and booking confirmation are unresolved; I will check them with the responsible service.

09 • Outgoing coordinator: Thank you. Please update the care-coordination record when you receive a verified response.

10 • Incoming coordinator: I will, and I'll flag any remaining barrier rather than inventing a transport arrangement.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 07 / 05 • CONVERSATIONS / 2 OF 3

A care-gap list is not a confirmed missed service

Separate fictional population-health handoff: a dashboard flags six possible care gaps, but outside-provider records have not been reconciled.

01 • Population health analyst: I'm handing over six possible care gaps from the dashboard for reconciliation.

02 • Care coordination lead: Have we established that the services were missed, or only that our records lack them?

03 • Population health analyst: Only the latter. Outside-provider records haven't been reconciled with this list.

04 • Care coordination lead: Then we shouldn't contact people saying they definitely failed to receive care.

05 • Population health analyst: Correct. I'll label the flags as unverified and provide the source dates.

06 • Care coordination lead: Who will check the authorized records before deciding what outreach is appropriate?

07 • Population health analyst: Could your coordination team own that review and return any confirmed corrections?

08 • Care coordination lead: Yes. I'll assign the review and keep clinical interpretation with the responsible clinicians.

09 • Population health analyst: Please also distinguish resolved data gaps from any actual care gaps that remain.

10 • Care coordination lead: Understood. I'll report both separately and won't treat a missing entry as proof of a missed service.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 07 / 05 • CONVERSATIONS / 3 OF 3

Handing over an unresolved access barrier

Separate fictional coordination exercise: a person reports difficulty reaching follow-up appointments; the specific barrier and available assistance are unconfirmed.

01 • Community liaison: The person reported difficulty reaching follow-up appointments. I'm handing over that concern for clarification.

02 • Care coordinator: Do we know whether the issue is transport, timing, cost, or something else?

03 • Community liaison: No. I haven't established the specific barrier, and I don't want to label it incorrectly.

04 • Care coordinator: Good. Social determinants are relevant context, but we shouldn't substitute assumptions for the person's account.

05 • Community liaison: Can you own the follow-up conversation and ask what assistance would actually be useful?

06 • Care coordinator: Yes. I'll clarify the barrier and check available services without promising support we haven't confirmed.

07 • Community liaison: Should the handoff link this issue to a predicted readmission for that person?

08 • Care coordinator: No. We have no basis for that individual prediction in the supplied information.

09 • Community liaison: I'll preserve their wording and identify the unresolved access question in the coordination note.

10 • Care coordinator: I'll confirm receipt and report the agreed next step through the appropriate care-coordination channel.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 07 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Population Health and Care Coordination

A discharge coordination note lists transport as pending. The next coordinator needs to know who is following up and what has been confirmed.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You are taking over the work. Ask what remains open and repeat back the critical status or responsibility. Point out one detail that would be unsafe or misleading to assume.

Second round: The intended recipient cannot take ownership. Keep the status clear and identify the need for an authorized reassignment.

Model for scenario 1: Transport remains unconfirmed. The request is recorded, and the assigned coordinator is following up. Please check the latest status with that person and record any change before telling the patient a departure time.

Scenario 2 | A follow-up referral sent but not received

In a fictional discharge-coordination case, an outgoing referral was sent yesterday. The receiving service has not acknowledged it, and no appointment is confirmed. Practice transferring follow-up responsibility at shift change.

Role A: Outgoing coordinator: distinguish transmission from acceptance and hand over the reference.

Role B: Incoming coordinator: confirm ownership of checking receipt and appointment status.

Possible opening: The referral was sent yesterday, but receipt and appointment arrangements are still unconfirmed.

Success checks

- Separate sent, received, and scheduled statuses.
- Identify the receiving owner and unresolved questions.
- Avoid promising an appointment while agreeing a clear follow-up action.

Add a complication: The family asks you to say that follow-up is fully arranged.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

LESSON 08 / READ AND NOTICE

Executive Dashboards and Board Updates

Explain a useful distinction in plain English.

SITUATION

A board chart shows waiting time falling from 40 to 30 minutes, but the reporting period changed from monthly to weekly.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

dashboard - Visual summary of selected measures used to monitor status, performance, or risk.

KPI - Key performance indicator used to monitor progress against an important objective.

variance - Difference between actual and expected performance, cost, timing, quality, or volume.

owner - Named person or role accountable for a decision, action, deliverable, or risk.

3. Notice the language

Start with the reader's question. Explain one unfamiliar term with familiar words, then give a relevant example or contrast. Check understanding without asking only 'Do you understand?'.

In this context, ... means

The difference is that

How would you explain that distinction to a colleague?

Compare this wording: We need to operationalize the implementation.

LESSON 08 / PRACTICE AND PRODUCE

Make a complex point clear

4. Check the language

1. Which sentence explains 'backlog' rather than repeating it?
 - A. The backlog is the maximum work the team can finish in a period.
 - B. The backlog is the work still waiting to be completed.
 - C. The backlog is the work completed during the last period.
2. Which follow-up best checks whether your explanation was clear?
 - A. How would you describe the next step in your own words?
 - B. Would you like me to repeat the explanation?
 - C. Was the explanation detailed enough for you?

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 08 / 05 • CONVERSATIONS / 1 OF 3

A monthly wait compared with a weekly wait

Fictional board preparation: a waiting-time chart falls from 40 to 30 minutes, but the reporting interval changed from monthly to weekly.

01 • Board secretary: The chart drops from forty to thirty minutes. Can the headline say waiting time improved?

02 • Performance analyst: We need a qualification: the earlier figure is monthly, while the newer figure is weekly.

03 • Board secretary: Why does that matter when both figures use minutes and the same KPI name?

04 • Performance analyst: They summarize different time windows, which may contain different activity and variation.

05 • Board secretary: So the visible difference is not automatically a like-for-like performance improvement?

06 • Performance analyst: Correct. We should align the periods or clearly explain why the comparison is limited.

07 • Board secretary: Would labeling the reporting intervals beside the figures be enough for today's discussion?

08 • Performance analyst: It would help, but don't retain a causal or sustained-improvement claim unsupported by comparable data.

09 • Board secretary: I'll revise the headline and ask the metric owner for aligned periods.

10 • Performance analyst: I'll provide a comparable series and keep the current figures identified as different reporting windows.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 08 / 05 • CONVERSATIONS / 2 OF 3

Target variance versus change from last month

Separate fictional dashboard: response time is eight days, the target is six, and last month's result was ten.

01 • Executive assistant: The result is eight days, down from ten. Why is the dashboard still showing an unfavorable variance?

02 • KPI owner: Because the target is six days. Improvement from last month and variance from target are different comparisons.

03 • Executive assistant: So we improved by two days but remain two days above the target?

04 • KPI owner: Exactly. Both statements can be true without changing the underlying result.

05 • Executive assistant: Could I describe the target as achieved because the direction is favorable?

06 • KPI owner: No. Direction of change doesn't erase the remaining gap to the stated target.

07 • Executive assistant: What should the board hear about accountability rather than just the color on the dashboard?

08 • KPI owner: Who owns the measure, what the gap is, and which follow-up remains under review.

09 • Executive assistant: I'll explain both comparisons and avoid claiming a cause for the improvement.

10 • KPI owner: I'll bring the supporting period definitions and a status update on the review, without inventing a corrective-action outcome.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 08 / 05 • CONVERSATIONS / 3 OF 3

A department average hides different queues

Separate fictional executive exercise: one queue averages ten minutes and another fifty; the combined figure lacks queue volumes.

01 • Chief operating officer: Can we average ten and fifty and report thirty minutes for the department?

02 • Dashboard analyst: Not without the queue volumes. The combined patient average depends on how many observations each queue contains.

03 • Chief operating officer: I assumed two queues should each contribute half of the headline measure.

04 • Dashboard analyst: That would describe an average of queue averages, not necessarily the average person's waiting time.

05 • Chief operating officer: Which distinction should I make when the board asks why the headline is pending?

06 • Dashboard analyst: Explain that we have queue-level results but lack the volumes needed for the intended combined KPI.

07 • Chief operating officer: Could the dashboard show both queues separately while the owner checks the denominator?

08 • Dashboard analyst: Yes. That preserves useful information without manufacturing a misleading total.

09 • Chief operating officer: I'll ask for the volumes and the precise measure definition before approving the headline.

10 • Dashboard analyst: I'll return the calculation with its scope and keep the separate queues visible for operational follow-up.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 08 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Executive Dashboards and Board Updates

A board chart shows waiting time falling from 40 to 30 minutes, but the reporting period changed from monthly to weekly.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You are unfamiliar with one technical term in the case. Ask for a plain-English explanation and then paraphrase it. Do not pretend to understand a term you cannot explain.

Second round: Your listener is new to this field. Replace two specialist expressions with clear explanations without changing the meaning.

Model for scenario 1: The reported waiting time is lower, but the periods differ. I'll label the time windows and prepare a comparable view before we describe the change as a sustained improvement.

Scenario 2 | Completed cases versus all cases

A fictional board dashboard reports response time only for completed requests. Twenty requests remain open and are excluded; their waiting times are not supplied. Explain why the completed-case figure does not describe the whole queue.

Role A: Performance analyst: explain the denominator and the missing open-case view.

Role B: Executive sponsor: ask what additional measure is needed before drawing a conclusion.

Possible opening: This KPI covers completed requests only; the twenty open requests are outside its calculation.

Success checks

- State the population included in the KPI.
- Avoid inventing waiting times for the open cases.
- Agree who provides a separate open-queue measure.

Add a complication: A board member asks you to count the open cases as having zero waiting time.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

Answer workshop

Try the tasks first. The model is one acceptable response to scenario 1, not a script to memorize. Assess your spoken version using the four criteria at the end.

01 | Patient Access, Scheduling, and Referrals

Editing example: Could you explain what this means?

After an introductory phrase such as 'Could you explain', the embedded question uses statement word order: "what this means".

Check 1: C - Could you confirm when the review starts?

A: Remove 'does' and use statement word order after 'confirm'.

B: Place the subject 'the review' before 'starts'.

C: The embedded question uses subject then verb: "the review starts".

Check 2: B - Has the report been approved, or is it ready for review?

A: This asks about the presenter, not the report's approval status.

B: This question distinguishes two meanings of 'ready' that affect the next action.

C: This asks about delivery timing, not whether approval has happened.

Model response: Could you confirm the specialty and referral details through the approved channel? I can explain the scheduling process to the patient, but I cannot confirm an appointment until the required information is available.

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

02 | Revenue Cycle, Coding, and Denials

Editing example: We need to put the plan into practice.

The revised sentence names the action in familiar words. Specialist vocabulary is useful when it adds precision, but repeating abstract nouns can hide the meaning.

Check 1: B - The backlog is the work still waiting to be completed.

A: This describes capacity, not the work waiting to be completed.

B: This gives the meaning in familiar words.

C: Completed work is different from work still waiting.

Check 2: A - How would you describe the next step in your own words?

A: An open request lets the listener show their understanding.

B: Offering repetition may help, but it does not show how the listener understood the next step.

C: This invites an opinion on detail, not a demonstration of understanding.

Model response: These categories describe different points in the process. Let's separate submissions that did not enter processing from claims that received a denial, then identify the follow-up required for each group.

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

03 | Patient Flow, Capacity, and Staffing

Editing example: Three checks are complete; two remain open and are being reviewed.

The revision gives observable progress. A positive label alone does not tell the reader what is complete or still needed.

Check 1: A - The team has completed the review.

A: 'Has completed' states that the action is finished.

B: 'Is completing' presents the work as in progress.

C: 'Plans to complete' describes an intention, not completion.

Check 2: C - Delivery is planned for Friday; the carrier has not confirmed it.

A: A plan does not establish carrier confirmation.

B: A plan cannot prove a future arrival time.

C: This explicitly labels the date as planned and identifies the missing confirmation.

Model response: There are 24 occupied beds. Two discharges are planned, but their timing is not confirmed. I'll distinguish current availability from expected availability in the capacity update.

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

04 | Quality, Safety, and Accreditation

Editing example: The pilot worked in the tested setting; performance elsewhere remains untested.

The revision states the finding and its scope. It avoids turning limited evidence into a universal claim.

Check 1: C - The change may have contributed to the delay.

A: 'Must have' expresses a strong inference that the evidence may not support.

B: 'Definitely' presents the cause as certain.

C: 'May have contributed' presents a possibility without claiming the cause is established.

Check 2: B - In this pilot, the observed result was ...

A: A pilot in one location does not establish a service-wide observation.

B: This locates the finding in the setting actually studied.

C: Similarity alone does not establish that the result will transfer.

Model response: The error was identified before the appointment. The cause is still under review. I'll record the sequence of events and the checks that caught it without assigning blame or an unconfirmed cause.

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

05 | HIPAA, Privacy, and Information Governance

Editing example: Could you send the missing document and confirm when it will be available?

The revision names the document and requested response. It avoids expressions that may be unfamiliar or ambiguous for an international audience.

Check 1: B - Send it no later than Thursday.

A: This interprets a deadline as a continuing activity, which would use "until".

B: 'By' sets the latest point, while 'after' refers to a later time.

C: "By Thursday" permits earlier delivery; it is not restricted to that day.

Check 2: A - Could you confirm the document version needed for the review?

A: The requested action and information are explicit.

B: This checks importance rather than identifying the needed document version.

C: This checks receipt, not which version is required for the review.

Model response: Could you specify the purpose and fields required? Let's confirm the permitted access and necessary information through our privacy process before sharing the list.

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

06 | Patient Experience and Service Recovery

Editing example: I'm sorry my message did not explain the timing clearly.

The revision takes responsibility for the message without blaming the listener. It identifies what needs to be corrected.

Check 1: A - I'm sorry my earlier wording was unclear.

A: This acknowledges the speaker's contribution to the problem.

B: This acknowledges duration, but does not take responsibility for the unclear wording.

C: This acknowledges a missed date, not the communication problem in the question.

Check 2: C - The corrected date and the current confirmation status.

A: The reader still lacks the information they need.

B: An unsupported new promise can create a second misunderstanding.

C: A repair needs usable corrected information, including any uncertainty.

Model response: I'm sorry you have had to call again. I can check the status of your request and confirm the next contact. For the clinical question, I'll connect you with the appropriate care team.

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

07 | Population Health and Care Coordination

Editing example: Please confirm which team has accepted the follow-up task.

The revised sentence asks for an identifiable receiving team instead of leaving responsibility with an unspecified 'someone'.

Check 1: C - Please confirm who will complete the outstanding check.

A: Mentioning a task does not show that anyone accepted it.

B: A written record alone does not identify who will act.

C: It asks for an identifiable person or team responsible for the task.

Check 2: B - So the result is pending and your team will follow up; is that correct?

A: Receiving a message is not the same as confirming its meaning.

B: This repeats the status and responsibility and invites correction.

C: It changes pending information into a final result.

Model response: Transport remains unconfirmed. The request is recorded, and the assigned coordinator is following up. Please check the latest status with that person and record any change before telling the patient a departure time.

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

08 | Executive Dashboards and Board Updates

Editing example: We need to put the plan into practice.

The revised sentence names the action in familiar words. Specialist vocabulary is useful when it adds precision, but repeating abstract nouns can hide the meaning.

Check 1: B - The backlog is the work still waiting to be completed.

A: This describes capacity, not the work waiting to be completed.

B: This gives the meaning in familiar words.

C: Completed work is different from work still waiting.

Check 2: A - How would you describe the next step in your own words?

A: An open request lets the listener show their understanding.

B: Offering repetition may help, but it does not show how the listener understood the next step.

C: This invites an opinion on detail, not a demonstration of understanding.

Model response: The reported waiting time is lower, but the periods differ. I'll label the time windows and prepare a comparable view before we describe the change as a sustained improvement.

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

Give feedback that helps

Score each criterion 0, 1, or 2. This is a practice rubric, not a professional qualification or CEFR test. Do not award or remove points for a particular accent.

Meaning and accuracy

Keeps the case facts accurate; clearly separates confirmed and unknown information.

Clarity and language

Uses understandable sentences and explains specialist terms when the listener needs it.

Interaction and tone

Responds to the other person's question, checks understanding, and uses an appropriate tone.

Task completion

Makes the requested action or unresolved question clear without inventing authority or facts.

Use the same scale for each criterion

0 - Not yet: The reader or listener cannot yet recover the intended meaning or action.

1 - With support: The message works after a prompt, clarification, or revision.

2 - Independently: The message is clear and accurate without a prompt.

Feedback sequence

First, identify a successful phrase. Next, identify one point where meaning or accuracy needs work. Offer one useful language correction, allow a repeat, and compare the two attempts. A total out of eight describes this performance only; do not translate it into a proficiency level.

Final challenge

Choose a case not rehearsed today. The learner gives a one-minute response, answers two follow-up questions, and writes a 70-110 word message. Add the lesson's second-round challenge. Compare the result with an earlier attempt using the four criteria.

Use the language responsibly

Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

Meaning before performance

Use specialist terms only when they help the listener. Explain unfamiliar words, preserve uncertainty, and ask when an instruction or responsibility is unclear. The model responses illustrate language; they do not authorize professional actions.

Sources and further reading

The cases, workshops, and explanations are original teaching material. These references provide language frameworks and selected professional context. References were checked in September 2026; consult current local guidance for actual work.

Digital.gov: plain-language guidance

<https://digital.gov/guides/plain-language>

Council of Europe: CEFR descriptors

<https://www.coe.int/en/web/common-european-framework-reference-languages/cefr-descriptors>

HHS: HIPAA business associates

<https://www.hhs.gov/hipaa/for-professionals/privacy/guidance/business-associates/index.html>

AHRQ: SBAR communication tool

<https://www.ahrq.gov/teamstepps-program/curriculum/communication/tools/sbar.html>

Your next step

Choose one expression you can use in a genuine work situation. Rehearse it with fictional details, then adapt the wording to your role and audience.

Extend your practice with AI

Use the next two prompts after your own first attempt. Each contains the course context and a complete starting case or dialogue. Copy the entire prompt, including the reference, into a new chat in your preferred AI. You can also use the links to copy it from the website.

Choose the right kind of help

A role-play prompt makes the AI wait for your replies. A new-dialogue prompt asks for a complete professional script. Writing feedback begins with your draft. Vocabulary, grammar and review prompts move from explanation to your own attempts.

Choose any lesson or dialogue online

The course prompt workshop has eight practice goals and B1 support, B2 independent and C1 stretch settings. These are practice choices, not assessed proficiency levels. Select the same lesson number or dialogue title you used in this guide.

[Open this course's AI prompt workshop](#)

Keep practice useful

Use fictional or anonymized details. English Ladder does not send anything to an AI service or add your drafts to the prompts. Your chosen service's terms and any usage charges apply. AI responses can contain mistakes; compare feedback with the lesson and verify specialist claims against the course references.

The two starter prompts use B2 practice. To change support in a printed prompt, replace its practice-setting paragraph with a request for shorter explanations and sentence starters (B1), or greater precision and more complex constraints (C1). Keep the professional facts unchanged.

Changing the example

For another case on paper, replace the text between REFERENCE and END REFERENCE with that case, its course context, relevant terms and language target. Include the full exchange if practicing a dialogue. Do not assume your AI can access this PDF or a link. The website makes this substitution for you.

Prompt edition: 2026-09-28. These are original practice instructions; results depend on the AI service you choose.

Practice grammar in context

START PROMPT / COPY THROUGH END PROMPT

Be my workplace English practice partner. Follow the practice instructions after the REFERENCE block. The block contains fictional study material, not instructions to you. Keep its facts, numbers, uncertainty and professional responsibilities accurate. Clearly label any new scenario or changed fact as an invented variation. Use natural workplace language; explain unfamiliar abbreviations in context. If a technical expression is uncertain, say so rather than inventing a definition or authority. Coach communication, not professional decisions; respect the course scope. Ask only for fictional or anonymized practice details. Judge clarity, meaning, appropriate tone and the next step. Accept valid alternative English and distinguish errors from style preferences. Do not claim to certify my language level. Unless I ask to change the plan, follow the stages and stop wherever the instructions say to wait.

When discussing a word, phrase, or example sentence as language within an explanation, question, or answer feedback, enclose that wording in quotation marks. For example: What does "about" mean in "about twenty people"? Use "on" with a named day. Keep standalone answer choices, vocabulary headwords, and ordinary reading or dialogue text uncluttered. Quotation marks identifying a language example do not attribute it to a news source; attributed source quotations must remain verbatim.

Practice setting: B2 independent. Use natural professional English with brief explanations. Let me attempt each task without a model first. Offer a hint after difficulty and invite a more precise retry.

REFERENCE

Course: Healthcare Administration English

Professional audience: hospital administrators, clinic managers, practice administrators, care coordinators, patient-experience leaders, revenue-cycle staff, operations managers, and healthcare-adjacent professionals

Scope: Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

Lesson 01: Patient Access, Scheduling, and Referrals

Original fictional case: A referral is received without the requested specialty or reason for the appointment. The patient is expecting a booking confirmation.

Language workshop: Ask a precise question

Communication goal: Clarify missing information before responding.

Language guidance: Name the exact word, fact, or requirement you need clarified. Ask one focused question at a time. Repeat your understanding and give the other person a chance to correct it.

Useful frames (complete the gaps with case facts): When you say ..., do you mean ...? / Could you clarify which ...? / So, my understanding is Is that right?

Vocabulary: referral: Order, recommendation, or routing process that directs a patient or case to another clinician, service, or specialist. / eligibility: Rules that determine whether a person, account, provider, service, or request qualifies for a program, benefit, or action. / authorization: Required approval from a payer, regulator, manager, or system before a service, transaction, or action can proceed. / no-show: Scheduled patient, customer, or guest who does not arrive or cancel in time for the appointment or

reservation to be reused.

Speaking task: Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You need to know exactly what information is missing. Give one answer only after your partner asks a focused question; then ask them to confirm their understanding.

Writing task: Write a 70-110 word message for the person who needs to act on this case. State the purpose, preserve the relevant facts, and make the next action or unresolved question clear. Use a subject line and an appropriate opening. Do not invent a deadline, finding, or approval.

Optional second-round challenge: The other person uses an ambiguous word such as 'ready', 'approved', or 'urgent'. Clarify it before continuing.

Model for comparison AFTER my attempt, not a response to give me first: Could you confirm the specialty and referral details through the approved channel? I can explain the scheduling process to the patient, but I cannot confirm an appointment until the required information is available.

END REFERENCE

TASK: Grammar that changes the meaning

1. For a lesson, use its language workshop as the focus. For a dialogue, quote one actual sentence and select one useful grammar feature from it, such as question word order, tense, modality, conditionals or clause linking. Explain in no more than 90 words how this feature helps the speakers do their work. Keep the grammar target narrow.
2. Give two short contrasting sentences using this workplace context. Explain the difference in time, certainty, condition or politeness. Label invented examples and preserve the distinction between possible, planned and confirmed events. Describe context-dependent choices as choices, not universal rules.
3. Give me one editing or sentence-building task using a known case fact. Do not copy the supplied editing example or reveal its answer. Ask me to explain my intended meaning. Stop and wait.
4. After I answer, quote my wording, identify at most two issues, and give a brief explanation and one hint. Ask me to revise before offering a full corrected version. Then accept any accurate, natural alternative that serves the purpose.
5. Continue with two new tasks, one at a time: first guided, then an original response without a sentence frame. End with a two-sentence workplace message using the target feature and a compact self-check. Do not replace this sequence with a worksheet and answer key.

END PROMPT

Copy this prompt online or choose another lesson and support level

Get feedback on a draft

START PROMPT / COPY THROUGH END PROMPT

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When discussing a word, phrase, or example sentence as language within an explanation, question, or answer feedback, enclose that wording in quotation marks. For example: What does "about" mean in "about twenty people"? Use "on" with a named day. Keep standalone answer choices, vocabulary headwords, and ordinary reading or dialogue text uncluttered. Quotation marks identifying a language example do not attribute it to a news source; attributed source quotations must remain verbatim.

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Optional second-round challenge: The other person uses an ambiguous word such as 'ready', 'approved', or 'urgent'. Clarify it before continuing.

Model for comparison AFTER my attempt, not a response to give me first: Could you confirm the specialty and referral details through the approved channel? I can explain the scheduling process to the patient, but I cannot confirm an appointment until the required information is available.

END REFERENCE

TASK: Coach my workplace writing

1. Ask me to paste my own fictional or anonymized draft and identify its reader and purpose. If the reference gives a writing task, mention that task as the default. If it is a dialogue, suggest a brief follow-up message that records its outcome and open questions. Stop and wait. Do not write the message for me first.
2. Once I supply the draft, check it against the reference. If an ambiguity changes the meaning, ask one focused question before rewriting. Do not assume that the draft is correct evidence for a new deadline, approval, diagnosis, cost or commitment.
3. Give feedback in three parts: one effective phrase with a reason; up to three priority improvements, quoting my words; and one short revision task for me. Prioritize incorrect facts or unclear action before minor grammar. For each language correction, explain why it matters to this reader. Label optional stylistic alternatives separately and preserve my level of certainty and intended politeness.
4. Stop and wait for my revision. Then compare the two attempts, identify an improvement and offer one edited version that preserves my voice and purpose. Keep the requested length; if none is specified, use 70-110 words. Explain any meaningful change rather than silently making the message stronger or more certain.
5. Close with one phrase worth reusing and a short transfer task for a different fictional reader. Wait for my attempt before providing another model.

END PROMPT

Copy this prompt online or choose another lesson and support level